

SAPPHIRE MEMORIAL MEDICAL CENTER

2000 Data Hub Pkwy, Austin, TX 78745 | (512)555-6603 | www.sapphiremmc.org

DISCHARGE SUMMARY

Patient Name: Daniyar Seitkali	DOB: June 18, 1965 (Age 60)
MRN: MRN-7241278	Gender: Male
Admission Date: August 22, 2025	Discharge Date: September 04, 2025 (13 days)
Attending Physician: Dr. Elspeth MacGregor, MD — Neurocritical Care	Insurance: Tricare #TRI-C3-33333
ICU Admission: YES — MICU/SICU	ICU Days: 9 of 13 total days

DIAGNOSES

Primary Diagnosis	ICD-10 Code	Status
Massive Pulmonary Embolism with RV Failure	I26.09	Acute / Critical
ESRD on Hemodialysis	N18.6	Chronic / Complicating
Morbid Obesity — BMI 52	E66.01	Chronic / Complicating

HOSPITAL COURSE

Daniyar, a 60-year-old male with a history of ESRD on Hemodialysis and Morbid Obesity — BMI 52, presented with confirmed massive bilateral pulmonary embolism with right ventricular failure and haemodynamic instability (SBP 74 mmHg). The patient was taken emergently to the cath lab where catheter-directed thrombolysis (CDT) with tPA was performed via bilateral pulmonary artery catheters. Haemodynamics stabilised within 6 hours. Follow-up echo at 48 hours showed RV recovery with RVSP 38 mmHg. Anticoagulation was transitioned to rivaroxaban. The patient was monitored in the cardiac ICU for 5 days before transfer to the general cardiology ward. Hypercoagulable workup was initiated and results pending at discharge.

VITALS AT DISCHARGE

■ NOTE: Several vital signs remain outside normal limits at time of discharge — close monitoring required.

BP	HR	SpO2	Temp	RR	Weight
91/58 mmHg	110 bpm	96% (on 2L O2)	99.6°F	22/min	83 kg

DISCHARGE MEDICATIONS

Medication	Dose	Frequency	Duration
Levetiracetam	1500 mg	Twice daily	Ongoing — neurology follow-up
Lacosamide	200 mg	Twice daily	Ongoing — neurology follow-up
Furosemide IV	80 mg	Twice daily	For diuresis — reassess at 1 week
Prednisolone	40 mg	Once daily	Taper over 3 weeks
Vancomycin	1.5g IV	Every 12 hours (AUC-guided)	14-day course
Meropenem	1g IV	Every 8 hours	10-day course — completed in hospital
Norepinephrine	Per protocol	Continuous IV infusion	Weaned and discontinued in hospital
Rivaroxaban	20 mg	Once daily with evening meal	Indefinite — PE management

FOLLOW-UP PLAN

URGENT: Neurocritical Care: Dr. Elspeth MacGregor, MD — within 72 hours of discharge (September 07, 2025).
Primary Care: Within 1 week (September 11, 2025). Full labs including BMP, CBC, LFTs.
Home health nursing: Daily vitals and wound check for first 7 days post-discharge.
Inpatient rehabilitation / LTACH: Transfer arranged — starts September 10, 2025.

■ **HIGH-RISK DISCHARGE — Patient requires close monitoring. Re-admission risk: HIGH.**

Attending Physician Signature: _____

Dr. Elspeth MacGregor, MD — Neurocritical Care | License #: TX-MD-43567 | Date: September 04, 2025

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